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The anatomy of a tertiary hospital

What it sells, what it costs, and what it takes to build.

Thirty four revenue lines, the cost stack behind them, and the same beds run three different ways. A hundred and fifty beds can bill NGN 1.0 billion a month or NGN 2.7 billion. The difference is strategy, not size, and it decides whether the capital comes back in three years or nineteen.

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A hundred and fifty beds, on the middle strategy.

Capacity		Activity a year	
Beds	150	Outpatient encounters	130,000
Occupancy	70%	Emergency attendances	26,000
Average length of stay	3.5 days	Admissions	10,950
Consulting rooms	24	Bed-days	38,325
Theatres	6	Theatre cases	3,400
ICU and HDU beds	20	Day cases and endoscopies	6,000
Dialysis, cath lab, CT, MRI, oncology	Yes	Laboratory tests	620,000
Laboratory, imaging, pharmacy	In house	Imaging studies	58,000

Across 166,950 patient encounters a year this hospital runs 3.7 laboratory tests and 0.35 imaging studies per encounter. Those two attach rates, more than any tariff, decide what it bills, far more than the bed count does.

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Same estate, same occupancy, three businesses.

	Low margin	Mid margin	High margin
Average length of stay	4.5 days	3.5 days	2.8 days
Admissions a year	8,517	10,950	13,688
Theatre cases	1,900	3,400	6,200
Day cases and endoscopies	2,400	6,000	14,000
Outpatient encounters	76,000	130,000	230,000
Laboratory tests	365,000	620,000	1,200,000
Revenue per occupied bed-day	NGN 306,754	NGN 499,052	NGN 858,687
Revenue a year, NGN M	11,756	19,126	32,909
Revenue a month, NGN M	980	1,594	2,742
EBITDA margin	13.5%	21.5%	25.8%
EBITDA a month, NGN M	**132**	**343**	**708**

A hundred and fifty beds producing a billion a month is not a size. It is a strategy, and it is the weakest one on this page. The same beds at the same occupancy run the third way bill 2.8 times as much and keep NGN 708 million a month instead of NGN 132 million, which is 5.3 times the profit on no extra bricks.

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Four decisions, and none of them is the bed count.

The decision	Low margin	High margin	What it does
How long a patient stays	4.5 days	2.8 days	Shorter stay, 5,171 more admissions through the same beds
How much the theatre runs	1,900	6,200	The biggest line in the building, and the one with capital behind it
Whether you treat without admitting	2,400 day cases	14,000 day cases	Volume that never touches a bed
Whether diagnostics serve outsiders	365,000 tests	1,200,000 tests	The highest margin in the building, sold beyond your own patients
Who pays	Bad debt 3.0%	Bad debt 1.2%	Payer mix shows up as margin, not just as cash

Revenue per occupied bed-day moves from NGN 306,754 to NGN 858,687, a factor of 2.8, with no extra beds and no extra occupancy. Every one of those four decisions is a management decision, not a capital one.

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Four sizes, three strategies, NGN M a month.

Revenue a month	Low margin	Mid margin	High margin
30 beds	196	319	548
50 beds	327	531	914
100 beds	653	1,063	1,828
150 beds	980	1,594	2,742

EBITDA a month	Low margin	Mid margin	High margin
30 beds	14 (7.0%)	48 (15.0%)	106 (19.3%)
50 beds	31 (9.5%)	93 (17.5%)	199 (21.8%)
100 beds	78 (12.0%)	213 (20.0%)	444 (24.3%)
150 beds	132 (13.5%)	343 (21.5%)	708 (25.8%)

A hundred beds run well bill NGN 1,828 million a month. A hundred and fifty run poorly bill NGN 980 million. Fifty beds run well bill NGN 914 million, which is what a hundred and fifty low-margin beds bill on three times the estate. Size is not the strategy.

The equipment still costs more than the building.

	Floor area	Building	Equipment	Land	Working capital	Total
30 beds	3,300 sqm	3,960	6,014	600	500	11,074
50 beds	5,000 sqm	6,000	9,408	900	800	17,108
100 beds	9,200 sqm	11,040	14,802	1,500	1,500	28,842
150 beds	13,200 sqm	15,840	20,336	2,000	2,000	40,176

NGN M. Building at NGN 1,200,000 a square metre, Lagos or Abuja. Equipment converted at NGN 1,550 to the dollar.

Published equipment prices, USD	New	Refurbished
CT, 64 slice	500,000 to 700,000	175,000 to 390,000
MRI, 1.5T	1,200,000 to 2,000,000	475,000 to 975,000
MRI, 3.0T	2,100,000 to 3,700,000	1,000,000 to 1,900,000
Cath lab, single plane	800,000 to 1,500,000	250,000 to 600,000
Cath lab, biplane	1,500,000 to 2,500,000	400,000 to 900,000

Five ways to put the same 150 beds on the ground.

Route	Capex, NGN M	Rent a year	What you give up
Greenfield, new equipment	40,176	none	Best specification, longest programme, heaviest cheque
Greenfield, refurbished imaging	35,702	none	Saves roughly a fifth of the equipment bill. Older platform, shorter service life
Convert an existing building	27,683	none	No land, cheaper shell, worse geometry and higher mechanical cost
Lease and fit out	23,881	620	Lowest cheque. Rent becomes a permanent charge against EBITDA
Partner the imaging	22,396	none	Partner funds CT, MRI and cath for a share of the studies. Capex down, margin down

The cheque moves from NGN 40,176 million to NGN 22,396 million, a 44 per cent swing, and every naira saved is bought with something: an older imaging platform, a worse building, a permanent rent, or a share of the highest margin line in the hospital.

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The same building pays back in 5 years or 25.

Years to repay the capex	Low margin	Mid margin	High margin
30 beds	67.3	19.3	8.7
50 beds	46.0	15.3	7.2
100 beds	30.7	11.3	5.4
150 beds	25.3	9.8	4.7

Identical bricks, identical beds, identical occupancy. 4.7 years against 25.3 years, decided entirely by length of stay, theatre throughput, how much diagnostics work the building takes in, and who pays. The capital committee argues about the first number. The result is set by the last four.

SENSE CHECK

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Where this model is safe, and where it is exposed.

Assumption	This model	The benchmark	Verdict
Revenue per occupied bed-day, top strategy	NGN 858,687	Netcare implies about NGN 947,874	Conservative
Revenue per occupied bed-day, low strategy	NGN 306,754	32 per cent of Netcare	A genuinely weak hospital
EBITDA margin, top strategy	25.8%	Netcare 18.6%, Life Healthcare 15.3%	Above both. The most exposed number here
Employee cost	34 to 38%	Netcare 38.7%	In line
Materials	28.8%	Netcare cost of goods 26.4%	Slightly above, for import exposure
Bad debt and rejected claims	1.2 to 3.0%	Netcare 0.8%	Deliberately worse. Nigerian payer reality
Occupancy	70%	Netcare 65.0%, Life 69.2%	Slightly optimistic
Imaging equipment prices	Published list	Block Imaging 2025 and 2026 guides	Sourced
Build cost	NGN 1,200,000 a sqm	Nigerian trade rates imply 300 to 400k commercial	Judgement. Not sourced
Capex a bed, 150 beds	USD 172,800	Aga Khan Uganda 667k, AMCE Abuja 1.5m	Far below. A commercial build, not a flagship
Margin penalty below 100 beds	1.5 to 6.5 points	Nothing published found	Judgement. Not sourced
Staff per bed	5.0	Tertiary norm 3 to 5	At the top of the range

Three numbers carry the argument and none is sourced: the build cost a square metre, the top-strategy margin, and the penalty for being small. Push on those three and the payback table moves. Everything on the revenue side sits below a listed comparator, so the case is not being made by optimistic billing.

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NGN 1.6 billion a month is NGN 52 million a day.

52 M
revenue a day

457
patient encounters a day

30
admissions a day

9
theatre cases a day

1699
laboratory tests a day

159
imaging studies a day

Put another way: every occupied bed in this hospital has to generate NGN 499,052 of total revenue a day, against a bed charge of NGN 28,000. The bed is the ticket. It is not the fare.

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Where the money actually sits.

Family	NGN M a year	A month	Share	What it is
Procedures	5,429	452	28.4%	Theatre, maternity, day case, dialysis, physiotherapy
Sellables	3,854	321	20.2%	Pharmacy, consumables, implants, gases
Diagnostics	3,747	312	19.6%	Laboratory, imaging, blood, histopathology
Stay	2,822	235	14.8%	Ward, critical care, neonatal
Consultation and access	2,137	178	11.2%	Clinics, emergency, telemedicine
Contracted and ancillary	1,137	95	5.9%	Checks, occupational health, retainers, concessions
Total	**19,126**	**1,594**	**100%**	**34 lines**



Diagnostics and sellables together are 40 per cent of the money, NGN 633 million a month, and a patient asked for almost none of it.

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All 34 lines, and the NGN 19.1 billion a year they add to.

Line	Volume	Price	NGN M	%
Theatre	3,400	720,000	2448.0	12.8
Laboratory	620,000	3,600	2232.0	11.7
Ward accommodation	38,325	45,000	1724.6	9.0
Pharmacy, inpatient	10,950	150,000	1642.5	8.6
Outpatient clinic	130,000	12,000	1560.0	8.2
Day case and endoscopy	6,000	180,000	1080.0	5.6
Critical care	5,475	180,000	985.5	5.2
Consumables	7,100	110,000	781.0	4.1
Cardiac catheterisation	550	1,400,000	770.0	4.0
Pharmacy, retail	117,000	6,500	760.5	4.0
Imaging, CT and MRI	10,000	60,000	600.0	3.1
Maternity	1,095	530,000	580.4	3.0
Implants and prostheses	340	1,600,000	544.0	2.8
Imaging, plain and ultrasound	48,000	10,000	480.0	2.5
Emergency	26,000	18,000	468.0	2.4
Executive health checks	2,200	200,000	440.0	2.3
Dialysis	5,400	42,000	226.8	1.2

Line	Volume	Price	NGN M	%
Corporate retainer	8,000	28,000	224.0	1.2
Oncology day unit	1,200	150,000	180.0	0.9
Cardiac diagnostics	7,800	20,000	156.0	0.8
Physiotherapy	12,000	12,000	144.0	0.8
Histopathology	3,600	40,000	144.0	0.8
Blood bank	3,000	45,000	135.0	0.7
Medical gases	14,000	9,000	126.0	0.7
Occupational health	3,000	40,000	120.0	0.6
Neonatal	1,314	85,000	111.7	0.6
Telemedicine	9,100	12,000	109.2	0.6
Vaccination and travel	6,000	16,000	96.0	0.5
Home care	1,800	40,000	72.0	0.4
Visa and immigration medicals	600	75,000	45.0	0.2
Medical reports	2,400	15,000	36.0	0.2
Space and concessions	180	200,000	36.0	0.2
Training and electives	120	300,000	36.0	0.2
Ambulance and transfer	320	100,000	32.0	0.2
Total, all 34 lines			**19,126**	

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NGN 178 M a month, 11.2 per cent of the total.

Line	The unit sold	Volume a year	Price, NGN	NGN M	Share
Outpatient clinic	Encounter	130,000	12,000	1560.0	8.2%
Emergency	Attendance	26,000	18,000	468.0	2.4%
Telemedicine	Session	9,100	12,000	109.2	0.6%
Family total				**2,137**	**11.2%**

Six per cent of the money, and the trigger for most of the rest. An encounter is not the product. It is the event that orders the laboratory, the scan, the drug and the admission.

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NGN 235 M a month, 14.8 per cent of the total.

Line	The unit sold	Volume a year	Price, NGN	NGN M	Share
Ward accommodation	Bed-day	38,325	45,000	1724.6	9.0%
Critical care	ICU or HDU bed-day	5,475	180,000	985.5	5.2%
Neonatal	Cot-day	1,314	85,000	111.7	0.6%
Family total				**2,822**	**14.8%**

The ward is under a tenth of revenue and the single largest fixed cost in the building. Critical care earns five times a ward bed-day and is what lets the hospital accept the complex case the rest of the list is built on.

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NGN 452 M a month, 28.4 per cent of the total.

Line	The unit sold	Volume a year	Price, NGN	NGN M	Share
Theatre	Case	3,400	720,000	2448.0	12.8%
Day case and endoscopy	Procedure	6,000	180,000	1080.0	5.6%
Cardiac catheterisation	Procedure	550	1,400,000	770.0	4.0%
Maternity	Delivery	1,095	530,000	580.4	3.0%
Dialysis	Session	5,400	42,000	226.8	1.2%
Oncology day unit	Chemotherapy cycle	1,200	150,000	180.0	0.9%
Physiotherapy	Session	12,000	12,000	144.0	0.8%
Family total				**5,429**	**28.4%**

The largest family, and the most capacity constrained. Three of these lines need capital before they need patients. A dialysis patient on a chair three times a week is worth NGN 6 million a year on his own.

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NGN 312 M a month, 19.6 per cent of the total.

Line	The unit sold	Volume a year	Price, NGN	NGN M	Share
Laboratory	Test	620,000	3,600	2232.0	11.7%
Imaging, CT and MRI	Study	10,000	60,000	600.0	3.1%
Imaging, plain and ultrasound	Study	48,000	10,000	480.0	2.5%
Cardiac diagnostics	Echo, ECG or stress test	7,800	20,000	156.0	0.8%
Histopathology	Specimen	3,600	40,000	144.0	0.8%
Blood bank	Unit	3,000	45,000	135.0	0.7%
Family total				**3,747**	**19.6%**

Almost none of this is requested by a patient. It is ordered, it carries the highest margin in the building, and it is the first thing to walk out of the gate when turnaround is slow or the machine is down.

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NGN 321 M a month, 20.2 per cent of the total.

Line	The unit sold	Volume a year	Price, NGN	NGN M	Share
Pharmacy, inpatient	Admission basket	10,950	150,000	1642.5	8.6%
Consumables	Procedure pack	7,100	110,000	781.0	4.1%
Pharmacy, retail	Walk-in basket	117,000	6,500	760.5	4.0%
Implants and prostheses	Device	340	1,600,000	544.0	2.8%
Medical gases	Unit	14,000	9,000	126.0	0.7%
Family total				**3,854**	**20.2%**

Good margin, and hungry for working capital. Stock-outs do not just lose the sale, they send the patient to the pharmacy outside and teach them to go there next time.

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NGN 95 M a month, 5.9 per cent of the total.

Line	The unit sold	Volume a year	Price, NGN	NGN M	Share
Executive health checks	Package	2,200	200,000	440.0	2.3%
Corporate retainer	Covered life	8,000	28,000	224.0	1.2%
Occupational health	Screening	3,000	40,000	120.0	0.6%
Vaccination and travel	Dose	6,000	16,000	96.0	0.5%
Home care	Visit	1,800	40,000	72.0	0.4%
Visa and immigration medicals	Examination	600	75,000	45.0	0.2%
Medical reports	Document	2,400	15,000	36.0	0.2%
Space and concessions	Square metre	180	200,000	36.0	0.2%
Training and electives	Place	120	300,000	36.0	0.2%
Ambulance and transfer	Trip	320	100,000	32.0	0.2%
Family total				**1,137**	**5.9%**

NGN 95 million a month from lines that need no bed and almost no scarce clinician time. Contracted, recurring, and mostly paid in cash on the day. This is the family most hospitals never build.

Four things fall straight out of the list.

- **Ordered, not chosen. NGN 633 million a month of it.**
  Diagnostics and sellables are 40 per cent of revenue and a patient requested almost none of it. The order sits with the clinician, which makes clinical governance a revenue function as well as a safety one.

- **Four lines each clear a billion a year on their own.**
  Theatre; Laboratory; Ward accommodation; Pharmacy for inpatients; Outpatient clinic and day case and endoscopy. Three of those four are consequences of the first: the theatre fills the bed, and the bed orders the drugs and the tests.

- **The bed charges NGN 45,000 a day and carries NGN 499,052.**
  Ward accommodation is 9.0 per cent of revenue. Forty beds carry a billion a month because the money is made around them, not in them.

- **NGN 95 million a month needs no bed and no scarce clinician.**
  Checks, retainers, occupational health, medicals and concessions. Contracted, recurring, paid on the day, and the family most hospitals never build out.

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What one more of each is actually worth.

One more	What it drags with it	All-in, NGN
Dialysis patient, a year	150 sessions at 42,000	6,300,000
Cardiac catheterisation	The lab, 2 bed-days, tests, drugs	1,668,800
Surgical case	Theatre, consumables, 3.5 bed-days, 14 tests, a scan, drugs	1,202,900
Oncology patient, a course	Six cycles at 150,000	900,000
Delivery	Suite or theatre, 3 bed-days, tests, drugs	803,800
Medical admission	3.5 bed-days, 14 tests, a scan, drugs	372,900
Executive check	A package. No bed, no scarce clinician	200,000
Outpatient encounter	Consultation, 3.5 tests, half a scan, a basket	36,100

A surgical case is worth about 33 outpatient encounters, and a catheterisation about 46. Filling theatre lists and cath lab sessions moves this hospital far harder than filling beds does, and a dialysis chair is an annuity.

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PART TWO

What it costs, and what it keeps

The same hospital from the other side: the cost stack, where the margin is actually made, and how far it sits from break-even.

What that revenue costs to produce.

	NGN M a year	A month	Of revenue	
Revenue	19,126	1,594	100.0%	34 lines
Direct materials	(5,500)	(458)	28.8%	Drugs, implants, reagents, contrast
Contribution	13,626	1,135	71.2%	
Employee costs, fully loaded	(6,694)	(558)	35.0%	Netcare runs 38.7%
Power and utilities	(574)	(48)	3.0%	Diesel, grid and solar. A Nigerian line
Equipment maintenance and service contracts	(612)	(51)	3.2%	CT, MRI, cath lab, theatre
Bad debt and rejected claims	(421)	(35)	2.2%	Netcare runs 0.8%. This is the gap
Repairs, cleaning, catering, security	(306)	(26)	1.6%	Hotel services
Administration, legal, audit	(344)	(29)	1.8%	Group and site
Marketing and business development	(383)	(32)	2.0%	
Insurance, licences and regulatory	(153)	(13)	0.8%	HEFAMAA, MDCN, indemnity
IT and systems	(172)	(14)	0.9%	Records, imaging, laboratory, billing
EBITDA	**3,967**	**331**	**20.7%**	

Below EBITDA	NGN M	Of revenue	
Depreciation and amortisation	(956)	5.0%	Netcare runs 5.1%
Finance costs	(861)	4.5%	Netcare runs 4.6%
Profit before tax	**2,150**	**11.2%**	**The capital takes two thirds of the EBITDA**

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Revenue and margin do not sit in the same families.

Family	Revenue	Materials	Contribution	Margin	Share of margin
Procedures	5,429	1,466	3,963	73%	29.1%
Diagnostics	3,747	899	2,848	76%	20.9%
Stay	2,822	339	2,483	88%	18.2%
Consultation and access	2,137	64	2,073	97%	15.2%
Sellables	3,854	2,505	1,349	35%	9.9%
Contracted and ancillary	1,137	227	910	80%	6.7%
Total	**19,126**	**5,500**	**13,626**	**71%**	**100%**

Sellables are NGN 3,854 million of revenue and keep only 35 per cent of it. Diagnostics are smaller on the top line and keep 76 per cent. Growing the pharmacy grows the turnover and the stock bill. Growing the laboratory grows the profit.

The first 71 per cent of the hospital pays the bills.

	NGN M a year	What it means
Fixed cost to open the doors	9,659	Payroll, power, maintenance, admin. Owed whether a bed fills or not
Contribution margin	71.2%	What each naira of revenue leaves after materials
Break-even revenue	13,558	71 per cent of what it bills today
Break-even occupancy	50%	Against 70 per cent today
EBITDA	**3,967**	**Everything earned above the break-even line**

Just 20 points of occupancy separate break-even from today, and those 20 points carry the entire EBITDA. That is the whole argument for utilisation: the hospital does not earn gradually, it earns at the top.

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Against two listed African hospital groups.

	Revenue	EBITDA margin	Occupancy	Employee cost	Materials	Bad debt
Netcare, FY2025	26,344 Rm	18.6%	65.0%	38.7%	26.4%	0.8%
Life Healthcare, FY2025	25,100 Rm	15.3%	69.2%	not split	not split	n/d
This hospital	**19,126 NGN M**	**20.7%**	**70%**	**35.0%**	**28.8%**	**2.2%**

The five point gap, named

Netcare EBITDA margin, FY2025	**18.6**
Materials cost more here	**-2.4**
Claims rejected and never paid	**-1.4**
A leaner payroll	**+3.7**
Power, and everything else	**+2.2**
This hospital	**20.7**

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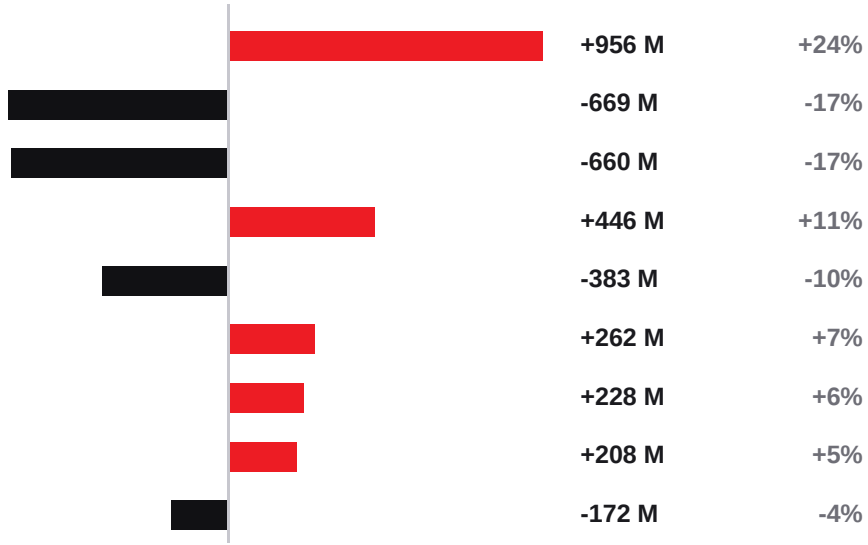
PART THREE

Variance, conversion, growth and risk

What moves the result, the cheapest growth in the building, how far a billion and a half is, and what breaks it on the way.

What actually moves the result.

Tariff, across the board	+5%	+956 M	+24%
Staff cost	+10%	-669 M	-17%
Naira devalues against imported materials	+20% on 60%	-660 M	-17%
Outpatient to inpatient conversion	+1 point	+446 M	+11%
Claims rejected and written off	+2 points	-383 M	-10%
Theatre cases	+10%	+262 M	+7%
Laboratory attach rate	+0.5 a patient	+228 M	+6%
Occupancy	+5 points	+208 M	+5%
Power and diesel	+30%	-172 M	-4%



Price is the largest single lever in the building and the cheapest to pull. Payroll and the exchange rate are the largest threats, and neither is inside management control. That asymmetry is the argument for pricing deliberately rather than annually.

You grow by treating people, not by admitting them.

The funnel	A year	Converts at	Where it goes
Outpatient clinic encounters	130,000		
Emergency attendances	26,000		
Admissions	10,950	7.0%	Capped by the bed, and the bed is the scarce thing
Procedures, theatre and day case	**11,150**	**7.1%**	**Capped by lists and rooms, not beds**

NGN 627 M

for every one point of procedure conversion

NGN 446 M

of that reaches EBITDA, a 11 per cent lift

The average procedure here is worth NGN 401,614 and the patient is already in the building, already examined, and often home the same day. Procedure conversion is capped by lists and recovery bays, which can be added. Admission is capped by beds, which cannot be, not without the capex on slide six.

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Moving from the middle strategy to the top one.

Lever	NGN M a year	What it needs
Length of stay, 3.5 days to 2.8	+821	Discharge planning, not more beds
Theatre, 3,400 cases to 6,200	+2,621	Lists, surgeons, anaesthesia, recovery
Laboratory, 620,000 tests to 1.2 million	+2,088	Outside referrals, turnaround, a courier network
Day case, 6,000 to 14,000	+1,440	Lists and recovery bays, not beds
Clinics, 130,000 to 230,000 encounters	+1,200	Rooms, sessions and specialists
Cath lab, 550 to 1,050	+700	A second interventionist
Everything inside this building	**+8,870**	

Those six moves add NGN 8,870 million, which is 64 per cent of the distance between the middle strategy and the top one. The high-margin column is not an aspiration. It is this list. The remaining NGN 4,913 million is what part four is about.

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What breaks it, and what it costs.

Risk	What happens	Cost if it lands
The naira	Implants, reagents and contrast are imported and priced abroad	NGN 660 M on a 20 per cent move
One clinician leaves	The cath lab, the oncologist, the lead surgeon. Capital strands with the person	NGN 455 M of lines sit on a handful of people
The payer stops paying	Rejection on technicality, then ninety day debtors	Every 2 points of write-off is NGN 383 M
A scanner goes down	Three weeks out and the referrers route elsewhere permanently	NGN 64 M of imaging, and the attach behind it
Power	Diesel is the swing cost and it is not hedgeable	NGN 172 M on a 30 per cent move
A sentinel event	A service line closes while it is investigated	The line, plus the referral pattern behind it
The replacement cycle	Depreciation is a real cash call every seven to ten years	NGN 956 M a year is being consumed

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PART FOUR

The other operating lines

Six levers close most of the distance to the top strategy. What closes the rest is a second business in the same building, and it earns on entirely different terms.

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What the other operating lines are worth.

Line	The unit sold	Volume	Price, NGN	NGN M	A month
Medical aesthetics	Treatment	7,000	250,000	1,750	146
Hair restoration	Procedure	500	1,800,000	900	75
Longevity and infusion	Session	10,000	85,000	850	71
Body contouring and metabolic	Programme	1,600	450,000	720	60
The cluster				**4,220**	**352**

	NGN M	Of revenue
Materials	(1,215)	28.8%
Staff	(928)	22.0%
Marketing and brand	(380)	9.0%
Device service, utilities, admin	(422)	10.0%
EBITDA	**1,275**	**30.2%**

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They look like healthcare and they do not behave like it.

	The hospital	The adjacent lines
Who decides to buy	A clinician, by order	The patient, by choice
Who pays	An HMO or a corporate scheme	The patient, on the day
When the cash arrives	Sixty to ninety days, conditionally	Immediately
What limits volume	Beds, theatre hours, scarce consultants	Rooms, devices and demand
What drives demand	Illness and referral	Brand, result and repeat visit
Cost of a new customer	Near zero. They arrive ill	Real. This is a marketing business
Exposure	Regulatory, clinical, payer	Discretionary spend and reputation
EBITDA margin	**20.7%**	**30.2%**

The hospital is the reason this works. Medical credibility, a theatre, sterilisation, a laboratory and an anaesthetist are exactly what the non-medical spa market cannot answer, and they are already paid for.

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The top strategy, and where it comes from.

	Revenue	A month	EBITDA	Margin
The hospital today	19,126	1,594	3,967	20.7%
The six levers	+8,855			
Additional fixed cost to deliver it			(2,100)	
The hospital, fully pushed	27,981	2,332	8,176	29.2%
The adjacent operating lines	+4,220	352	1,275	30.2%
The group	**32,201**	**2,683**	**9,451**	**29.3%**

The cluster is 13 per cent of group revenue and 13 per cent of group EBITDA. It is the smaller business and the better one, and it is what carries the group margin from 20.7 to 29.3 per cent.

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The second business fixes the first one's cash.

	Debtor days	Why
The hospital, fully pushed	75	Claims verified, queried, and paid in arrears
The adjacent lines	2	Paid at the desk before the patient leaves
The group, blended	**65**	**Weighted by revenue**

10 days

off the group's collection cycle

NGN 844 M

of working capital released, without borrowing it

A hospital can be profitable on paper and insolvent on cash. A cash-paying business inside the same group is the cheapest hedge against that, and it is worth more than the margin it reports.

It fails in ways the hospital never does.

- **It is discretionary spend.**
  Illness is not optional and a facelift is. The cluster is the first thing cut in a squeeze, which is exactly when the hospital needs it most.

- **It is a brand business run by clinicians.**
  The skills that fill a theatre list do not fill an aesthetics diary. This needs marketing, merchandising and a front desk that sells, and most hospitals are bad at all three.

- **The devices date faster than the building.**
  Lasers and platforms are obsolete in five to seven years and the replacement is not optional. Depreciation here is a real and recurring cash call.

- **A complication is a social media event.**
  Not an incident form. The reputational mechanics are consumer, immediate and public, and they reach the hospital's name as well.

- **It must not be allowed to distract the core.**
  The moment theatre time, nursing or management attention is pulled from the hospital to feed the cluster, the group has traded a 30.2 per cent margin for a 20.7 per cent one and gone backwards.

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Seven numbers that run the whole list

1	**Clinician activity**	Sessions delivered against sessions available.
2	**Attach rate**	Tests, images and items per encounter, by specialty.
3	**Internal fulfilment**	The share of orders the building filled itself.
4	**Theatre utilisation**	Cases against list hours, not against theatres owned.
5	**Occupancy and length of stay**	Read together. Either one alone misleads.
6	**Revenue per encounter**	By specialty. It exposes the mix without a cost model.
7	**Payer mix and debtor days**	Margin means nothing if the cash is ninety days out.